

Which structure is turning inward?

Entropion is lid rotation; trichiasis is lash misdirection with a normally positioned lid.

Compare the anatomy

Condition	Position	Consequence
Entropion	Lid margin rotates inward	Normally directed lashes contact the eye.
Trichiasis	Lid normal; lashes misdirected	Abnormal lashes abrade the ocular surface.
Epiblepharon	Redundant lower-lid skin fold	The fold directs lashes toward the eye.
Ectropion	Lid and punctum turn outward	Exposure and impaired tear drainage.

Treat the surface and the cause

Check for epithelial injury when lashes contact the eye or exposure causes irritation. Lubrication can protect the surface, but persistent malposition may need definitive correction. Watering does not mean the surface is adequately protected: an everted punctum drains poorly, and exposure can still leave the eye uncomfortable.

Ptosis can obstruct developing vision

A child's drooping lid needs assessment of whether it blocks a clear retinal image.

What does the ptosis affect?

Assess the lid, visual axis and visual function.

Pupil / visual axis obstructed

Deprivation amblyopia is a concern; arrange early ophthalmic assessment.

Visual axis clear

Still assess acuity, refraction and ocular alignment rather than judging appearance alone.

Associated diplopia or neurological signs

Look beyond an isolated congenital or aponeurotic lid problem.

Age and mechanism guide interpretation

Congenital ptosis can reflect abnormal development of the levator muscle. In older adults, aponeurotic ptosis is associated with stretching or dehiscence of the levator aponeurosis. These patterns differ from neurological or fatigable ptosis. The immediate childhood priority is visual development, especially when the pupil is covered or the eyes receive unequal image quality.